Perceived Parental Overprotection is Associated with Internalizing Symptoms Among Children of 9/11 First Responders

Daniel Griggi[1], Daniel Craft[1], Yael M. Cycowicz[1,2], Diana V. Rodriguez-Moreno[1,2]
Neuro Cognition and Emotion Lab
[1]New York State Psychiatric Institute, New York, New York
[2]Department of Psychiatry, Columbia University, New York, New York

INTRODUCTION

- Trauma-exposed parents are at risk of engaging in more maladaptive parenting behaviors than non-exposed parents. [1]
- Maladaptive parenting increases the likelihood of a child developing psychopathology that may persist into adulthood. [2]
- Authoritarian parenting practices have been identified as potential mediating factors among these maladaptive behaviors. [3]
- Parental overprotection reflects a dimension of authoritarianism in which excessive control and harsh discipline limit a child's freedom and independence. [4]
- Perceptions of parental overprotection are associated with various forms of behavioral pathology; however, existing literature suggests externalizing problems (e.g., substance and alcohol use disorders) often arise alongside concurrent internalizing problems. [5]

AIM

To assess the effect of parenting style by exploring the relationship between perceived parental overprotection and internalizing/ externalizing disorders in now-adult children of first responders who experienced significant PTSD symptoms following the 9/11 attacks.

HYPOTHESIS

We hypothesized that now-adult children with high scores on internalizing disorders would report greater perceived parental overprotection compared to those with high scores on externalizing disorders or no indication of mental health issues.

METHODS

Sample

- 117 now-adult children (born between 1980-2004) of 9/11 first-responder parents (see Table 1 for complete demographics).
- All parents reported significant symptoms of PTSD following 9/11.

Measures

- Self-report web-based survey using REDCap.
- Perceived parental overprotection was based on the following question: “Did your parent cut down on your freedom to go where you wanted to go?” (Yes/No)
- Mental health disorders were assessed using standardized instruments:
  a. Internalizing:
     i. Depression – PHQ-8
     ii. Anxiety – GAD-7
     iii. Panic – PDSR
  b. Externalizing:
     i. Alcohol Use (AUD) – AUDIT
     ii. Substance Use (SUD) – DAST

Analysis

- Based on mental health assessment scores, participants were sorted into internalizing, externalizing, and control groups:
  a. Internalizing group - participants scored above cut-off on one or more of the internalizing disorders and below cut-off on externalizing disorders.
  b. Externalizing group – participants scored above cut-off on one or both of the externalizing disorders and below cut-off on internalizing disorders.
  c. Control participants scored below cut-off on all disorders.
- A univariate analysis of variance was used to evaluate the impact of perceived parental protection on psychopathology presentation.

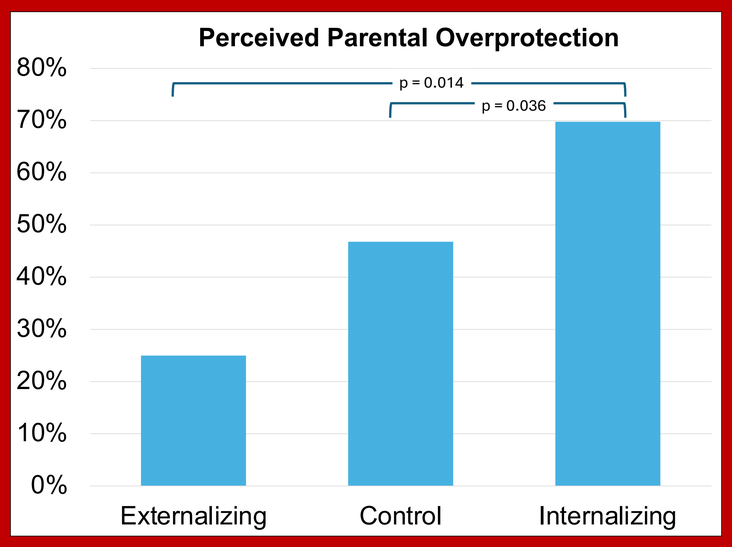
Main Findings

- Perceptions of parental overprotection are more strongly associated with the development of internalizing disorders, such as anxiety and depression, while having a marginal or inverse effect on externalizing disorders, such as substance use.
- The study highlights the importance of considering family dynamics and the potential impact of trauma on parenting styles, as these factors may play a role in the psychological development of children and the emergence of these disorders.

Main Results

Classification of Participants by Disorder Scores

	Internalizing Group (*n* = 46)		Externalizing Group (*n* = 12)		Control Group (*n* = 59)	
	M	***SD***	***M***	***SD***	***M***	***SD***
PHQ-8	9.89	6.65	3.67	2.67	3.03	2.97
GAD-7	11.39	5.36	3.67	2.23	2.41	2.59
PDSR	10.57	7.67	0.17	0.39	0.69	1.89
AUDIT	4.89	4.38	10.92	2.23	2.76	2.17
DAST	0.33	0.85	0.67	1.07	0.25	0.51



Association of Overprotection with Psychopathology Types

	SS	***df***	***MS***	***F***	**Sig.**	η_p^2	R^2
Groups	2.390	2	1.195	5.092	**.008***	.082	.082

ACKNOWLEDGMENTS
- NIOSH/CDC U01OH012065, World Trade Center Health Program (WTCHP) and General Responder Data Center (GRDC)

CONTACT:
dlg2176@tc.columbia.edu

Table 1. Demographics

		N (117)	%
Biological Sex	**Male**	50	42.0%
	Female	69	58.0%
Ethnicity	**Non-Hispanic**	78	65.5%
	Hispanic	38	31.9%
Race	**White**	82	86.9%
	Non-white	34	28.6%
Education	**Less than college**	42	35.3%
	College and above	76	63.9%
Marital Status	**Married**	40	33.6%
Age Range (Years)		21-43	

RESULTS

- Participants reporting restricted freedom had higher scores in anxiety, depression, and panic.
- Higher AUD and SUD scores were associated with an absence of perceived overprotection.
- Perceived parental restriction was most prevalent in the internalizing group (69.7%) followed by the control (46.8%) and the externalizing (25%) groups.
- The univariate analysis of variance revealed a statistically significant difference in levels of reported overprotection among groups ($F(2,114) = 5.092$, $p = .008$).
- Post-hoc tests indicate significant differences in parental overprotection between internalizing and both externalizing (M = -.45, p = .014) and control (M =.24, p = .036) groups. The externalizing group reports less overprotection than the control group, yet not significant at the .05 level.

CONCLUSIONS

- The findings suggest that perceptions of parental overprotection among now-adult children of 9/11-exposed first responders are more strongly associated with internalizing disorders, while potentially having an inverse effect on externalizing disorders.
- The absence of objective measures for parenting style and overprotection makes it difficult to determine whether participants’ current mental health influences perceptions of parenting style.
- Although overprotection appears to be associated with internalizing symptoms, the reported lack of overprotection in the externalizing group and the balance reported in the control group suggest that an appropriate or normative level of protection may be necessary for children’s well-being.
- This research implies that trauma exposure’s effect on parenting style and family dynamics plays a significant role in the development of internalizing and externalizing disorders in children, emphasizing the importance of incorporating these factors into trauma response services.

REFERENCES

1. Mowder, B. A., Guttman, M., Rubinson, F., & Sossin, K. M. (2006). Parents, Children, and Trauma: Parent Role Perceptions and Behaviors Related to the 9/11 Tragedy. *Journal of Child and Family Studies, 15*(6), 733-743.
2. Yahav, R. (2006). The relationship between children’s and adolescents’ perceptions of parenting style and internal and external symptoms. *Child Care Health and Development, 33*(4), 460-471.
3. Francis, S. E., Roemhild, E. (2022). Adolescent Perceptions of Maternal and Paternal Autonomy Granting and Psychological Control in Predicting Anxiety and Depression. *Marriage & Family Review, 58*(8), 781-810.
4. Banica, I., Sandre, A., & Weinberg, A. (2019). Overprotective/authoritarian maternal parenting is associated with an enhanced error-related negativity (ERN) in emerging adult females. *International Journal of Psychopathology, 137*, 12-20.
5. Gere, M. K., Villabø, M. A., Torgersen, S., & Kendall, P. C. (2012). Overprotective parenting and child anxiety: The role of co-occurring child behavior problems. *Journal of Anxiety Disorders, 26*(6), 642-649.